Bipartisan Policy Center

VIA ELECTRONIC SUBMISSION

June 13, 2025

The Honorable Dr. Mehmet Oz
Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
8013, Baltimore, MD 21244-8013

Re: Request for Information; Health Technology Ecosystem [CMS-0042-NC]

Dear Administrator Dr. Oz:

The Bipartisan Policy Center appreciates the opportunity to submit comments in response to the Centers for Medicare and Medicaid Services' request for information on CMS-0042-NC, "Health Technology Ecosystem."

BPC and its advocacy affiliate, Bipartisan Policy Center Action (BPC Action), are unique in their approach to addressing the nation's most pressing issues. As the only organization working across the full political spectrum on domestic issues, BPC brings together diverse perspectives to craft data-driven, pragmatic policy solutions. BPC Action then works directly with legislators and other policymakers to turn those solutions into real change.

For more than 20 years, BPC and BPCA have been consistently achieving results on complex issues, demonstrating that bipartisan cooperation is not only possible but essential for lasting solutions.

BPC's Health Program advances bipartisan policy solutions to build a more cost-effective, evidence-based health care system to improve population health. We work to strengthen and sustain Medicare and Medicaid, accelerate the shift toward value, address inefficiencies and misaligned incentives, and responsibly leverage technology and innovation.
Our comments reflect BPC's extensive work on Medicare Part B physician payment reform and digital health. Responses to this request for information include opportunities to improve electronic health record (EHR) system interoperability and enable access to high-quality digital products. Relevant BPC resources include:

- The Need for Medicare Part B Physician Payment Reform

- Maximizing the Value of Remote Patient Monitoring
- From Promise to Best Practice: The Realities of Remote Patient Monitoring
- The Future of Remote Patient Monitoring
- Positioning Telehealth Policy to Ensure High-Quality, Cost-Effective Care

Our comments reflect BPC staff views and do not necessarily represent the official positions of BPC's founders or board of directors.

This letter includes BPC's comments on select provisions of CMS' proposed rule, and the numbering system throughout this document reflects that of the proposed rule. While BPC did not comment on every provision in the proposed rule, such omissions should not be interpreted as a lack of interest in the proposals.

Patients and Caregivers

1. Patient Needs

PC-1. What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?
a. What are the top things you would like to be able to do for your or your loved ones' health that can be enabled by digital health products?
b. If you had a personal assistant to support your health needs, what are the top things you would ask them to help with? In your response, please consider tasks that could be supported or facilitated by software solutions in the future.

Patients and caregivers benefit from digital tools such as remote patient monitoring that offer real-time, actionable insights into their health. Remote monitoring solutions show promise in improving clinical outcomes and reducing costs, when used appropriately. These devices enable patients to send real-time data to their providers, who can remotely help them manage conditions such as hypertension, diabetes, musculoskeletal disorders, and mental health conditions.

Ebony Ford's story illustrates the potential of remote monitoring. In 2017, Ebony was 25 weeks pregnant when she developed symptoms of pre-eclampsia. She eventually underwent an emergency c-section and was placed on life support. During her next pregnancy, Ebony chose a new health care team that prescribed a home blood pressure cuff, enabling Ebony's doctors to monitor her vitals, adjust medication in real time, and promptly advise her when to seek further care. Ebony credits her healthier second pregnancy to having access to this virtual support and her remote blood pressure cuff.

Artificial intelligence-driven health apps could further support Medicare beneficiaries in managing chronic conditions. These apps could aggregate a patient's medical history, medications, and wearable data to provide tailored alerts (e.g. to take medications or schedule preventive services) to patients and caregivers.

For additional details, see BPC's video, Maximizing the Impact of Digital Health Technology.

PC-5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?
a. What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process? What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?
b. What changes would enable timely access to high quality CMS and provider generated data on patients?

To enable timely access to high-quality data, CMS should require remote patient monitoring (RPM) claims to include key details on the type of monitoring. A recent report by the Health and Human Services Office of Inspector General (OIG) identified data gaps that hinder CMS's ability to effectively oversee remote monitoring services. Medicare currently does not track the type of RPM devices used and the specific health data collected (e.g., blood pressure, respiratory flow rate). These data are key to understanding when and how these solutions improve patient outcomes and reduce costs. Given the diversity of tools and conditions covered under remote monitoring, CMS should be able to distinguish between different types of monitoring.

Additionally, Medicare often lacks information about the providers involved in remote monitoring, as it does not require ordering providers to include their identification number on claims. "Incident to" billing, where clinical staff bill under a supervising physician's national provider identification number, further complicates service quality monitoring and can obscure fraud detection. Both MedPAC and OIG have raised concerns about "incident to" billing across health care services. CMS should require that providers eligible for direct Medicare billing submit claims under their own identification number, rather than using "incident to" billing. At a minimum, CMS should be able to identify and monitor this type of billing.

For additional details, see BPC's reports, Maximizing the Value of Remote Patient Monitoring and The Future of Remote Patient Monitoring.

PC-6. What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

Making digital tools, such as remote monitoring devices, accessible and easy to use for Medicare beneficiaries and caregivers is essential to their success in improving outcomes.

There are several strategies and features that can be applied throughout the treatment process to achieve this goal.

Clear, published information on approved devices—including labels with details on how devices perform across subpopulations—would help beneficiaries and caregivers make informed choices. Choosing the best device can be challenging due to a lack of information regarding the effectiveness of devices. Even FDA-regulated devices may vary in accuracy and perform differently across demographic groups. For example, research has shown that pulse oximeters can be less accurate on people with darker skin.

To ensure usability, products should feature intuitive design, clear instructions available in multiple languages, and compatibility with smartphones via Bluetooth to minimize broadband barriers. Patients often benefit from technical assistance and adherence reminders, yet many providers lack capacity for consistent follow-ups. Care teams, including community health workers and digital navigators, often help provide this assistance. Additionally, generative artificial intelligence can help facilitate communication, such as through chatbots that troubleshoot technical issues.

CMS could offer technical assistance to help providers establish remote monitoring protocols that effectively engage patients, manage and secure incoming data, and respond to anomalies within a reasonable timeframe.

For additional details, see BPC's blog post, From Promise to Best Practice: The Realities of Remote Patient Monitoring, and explainer, Maximizing the Value of Remote Patient Monitoring.

PC-7. If CMS were to collect real-world data on digital health products' impact on health outcomes and related costs once they are released into the market, what would be the best means of doing so?

CMS could evaluate how digital health products perform in practice, to inform coverage and quality improvement. CMS can leverage existing data streams, including Medicare claims, hospitalization rates, and patient outcome metrics to compare cohorts who use a given digital tool versus those who do not (while adjusting for risk factors). To support the analysis, CMS could incentivize providers who adopt digital health products to submit outcome metrics for a period of time post-launch, potentially leveraging functionality within the tools to ease the reporting burden.

Data Access and Integration

PC-8. In your experience, what health data is readily available and valuable to patients or their caregivers or both?

a. What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?
b. What are specific sources, other than claims and clinical data, that would be of highest value, and why?
c. What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?

To improve the accessibility and interoperability of clinical data to support meaningful research, CMS and ASTP/ONC should collaborate with HRSA to inform efforts to simplify and consolidate similar or duplicative federal grant programs designed to help clinicians, particularly small and rural (non-hospital-affiliated) clinicians, adopt certified electronic health record technology (CEHRT).

With input from CMS, ASTP/ONC, and HRSA, HHS should then report on and make recommendations to Congress on any programs that require legislation for consolidation to improve administrative efficiency, while ensuring there are no reductions to appropriated funds.

Consolidating funding programs that are available to independent rural or small primary care practices is critical to avoiding duplication and inefficiencies while supporting CEHRT uptake. This approach aligns with the priorities of the administration.

In addition to enabling meaningful research, this focused streamlining and consolidation could also simplify the data reporting processes that are inefficient and administratively burdensome to providers. This would, in turn, leave providers with more time to deliver patient care and, in reducing the inefficiencies of non-interoperable EHR systems, enhance the types of clinical data sharing that lead to improved patient health outcomes.

BPC's forthcoming report, Strengthening Primary Care: Medicare Physician Payment and Related Reforms, will further examine these challenges and offer federal policy solutions.

PC-10. How is the Trusted Exchange Framework and Common Agreement™ (TEFCA™) currently helping to advance patient access to health information in the real world?
a. Please provide specific examples.
b. What changes would you suggest?
c. What use cases could have a significant impact if implemented through TEFCA?
d. What standards are you aware of that are currently working well to advance access and existing exchange purposes?
e. What standards are you aware of that are not currently in wide use, but could improve data access and integration?
f. Are there redundant standards, protocols, or channels that should be consolidated?

g. Are there adequate alternatives outside of TEFCA for achieving widespread patient access to their health information?

TEFCA represents a critical step toward building a unified, national approach to health information exchange. It shows considerable potential in addressing longstanding barriers to seamless patient health information exchange across providers, payers, and patients. However, the Agreement needs stronger alignment with CMS-led payment reforms and broader health system transformations to fully realize its promise. As AI becomes more prominent in the health care technology landscape, its inclusion in interoperability standards will be necessary.

BPC's Health Program is continuing to explore opportunities to strengthen interoperability across the health care system, including potential reforms to fully realize the potential of the TEFCA framework to advance the goals of value-based care. In the coming year, BPC will release a report with relevant recommendations to advance more seamless data sharing and to support better care coordination and patient outcomes.

Providers

1. Digital Health Apps

PR-1. What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients?
a. What are the current obstacles?
b. What information should providers share with patients when using digital products in the provision of their care?
c. What responsibilities do providers have when recommending use of a digital product by a patient?

There are multiple barriers to providers adopting high-value digital products. Establishing a high-quality remote monitoring program demands significant investments in devices, new technology infrastructure, and adapting care workflows—often hurdles for under-resourced facilities already operating under tight budgets.

Medicare's current reimbursement structure, however, does not account for these challenges. Average Medicare reimbursement for these services has decreased at a faster rate than overall reductions in the Physician Fee Schedule conversion factor, partially because the practice expense (PE) methodology may not fully capture the costs of software-based technologies. Additionally, geographic adjustments further reduce reimbursement for rural providers. These adjustments are not appropriate for digital tools, as deployment costs for such tools generally do not vary by location, and they disincentivize national companies from offering digital solutions in rural areas. Medicare's billing rules are another obstacle to adoption of remote

monitoring tools. Rigid requirements, such as requiring 16 days of data per month, often misalign with clinical needs and make reimbursement more difficult for providers.
To encourage more providers to leverage high-value remote monitoring tools for their patients, CMS should eliminate geographic adjustments that reduce payments to providers serving rural and lower-cost areas, ensure reimbursement levels are adequate overall, and simplify billing requirements that create administrative burden without adding value. Additionally, remote monitoring services align well with risk-based models – such as bundled payment or partial capitation – that link reimbursement to outcomes. These models naturally incentivize providers to focus on effective patient targeting and higher-value applications of digital tools.

Further, CMS should require providers to periodically reassess whether remote monitoring remains beneficial for their patients. Evidence suggests remote monitoring is most effective when used for a focused period of active monitoring. A recent OIG report found that one in four Medicare patients who received remote monitoring did so for longer than nine months. While extended monitoring may be appropriate for some patients, it often has limited value for those with well-controlled conditions. For example, most medication adjustments for hypertension occur within the first four months of monitoring. In traditional Medicare, where fee-for-service payments dominate, providers may have financial incentives to prolong monitoring beyond their clinical benefit. Medicare should require periodic reassessments to ensure it is covering these services only as long as patients need them.

For additional details, see BPC's report, Maximizing the Value of Remote Patient Monitoring.

PR-2. What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

Digital health applications that support physician workflows face several barriers to development and use. Some tools do not effectively integrate with EHR systems due to limited interoperability, lack of a clear regulatory or liability framework for AI-generated content, and CMS reimbursement and quality reporting programs that do not adequately reward providers for adopting tools that increase efficiency. While some of these tools can improve the accuracy of clinical documentation and billing processes, which can lead to better care and quality reporting, they often require upfront investments in both technology and workflow design. Without policy incentives to encourage both the development and adoption of these tools, stakeholders may be hesitant to invest the necessary resources.

PR-3. How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)? Please address all of the following:
a. Current challenges in accessing different data formats.
b. Impact on patient care quality.

c. Technical barriers to full data accessibility.
d. Cost or privacy implications of making all data formats interoperable.
e. Priority level compared to other interoperability needs.

Ensuring that all data within an EHR system—regardless of format—is accessible for exchange is essential to advancing health care delivery and interoperability in both urban and rural areas. This is especially important given CMS' interest in advancing value-based, preventive models of care and corresponding requirements that participants in alternative value-based payment models use certified EHR technology (CEHRT).

Despite some progress in promoting interoperability, many EHR systems have yet to achieve the critical mass necessary to fully realize the potential of value-based payment models. Clinicians in rural communities, in particular, experience acute barriers to accessing and sharing CEHRT data. This is often due to limited resources, reduced access to technical infrastructure, and workforce shortages that already constrain care delivery and hamper clinicians' efforts to participate in value-based models of care. When clinicians lack timely access to complete patient information, fragmented and inefficient care can result.

Not only can this be costly, but it undermines the goals of value-based care and, more specifically, hinders the CMS Innovation Center's efforts to unlock data access to improve quality and reduce health system costs.

BPC's Health Program is continuing to explore opportunities to strengthen EHR interoperability. In the coming year, BPC will release a report with relevant recommendations to advance more seamless accessible data sharing.

2. <u>Data Exchange</u>

PR-7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem? How can the burden of increasing data availability and sharing be mitigated for providers? Are there ways that workflows or metrics that providers are already motivated to optimize for that could be reused for, or combined with, efforts needed to support interoperability?

To support high-quality, timely data exchange and reduce provider burden, CMS should prioritize the standardization and alignment of health care quality measure data—particularly in primary care—across payers and models.

Today's fragmented quality measurement reporting requirements often require clinicians to report slightly different versions of similar metrics across payment models and programs, wasting time and resources as well as discouraging participation in value-based models. Aligning reporting requirements for these quality measures would ease administrative

burdens and enable clinicians to focus on care delivery rather than navigating redundant reporting tasks.

CMS should build on initiatives like the National Quality Strategy, the Universal Foundation for Quality Measures, and the Core Quality Measures Collaborative to develop standardized metrics that leverage structured data already captured in EHRs during routine care—such as vitals, labs, and medications. By designing metrics around existing workflows and actionable clinical information, CMS can promote interoperability and data availability without imposing additional documentation requirements on providers.

BPC's forthcoming report, Strengthening Primary Care: Medicare Physician Payment and Related Reforms, will further examine these challenges and offer federal policy solutions.

Payers

PA-5. What are ways payers can help with simplifying clinical quality data responsibilities of providers?

Payers can simplify clinical quality data responsibilities of providers by ensuring any billing requirements that create administrative burden add value. Certain Medicare billing rules are misaligned with clinical need and make reimbursement more challenging.

For instance, Medicare requires that patients collect at least 16 days of data per month for providers to receive reimbursement. However, the amount of data that providers need varies by condition; for example, providers might only need a few days of foot temperature data to monitor diabetic patients at risk of developing foot ulcers. Additionally, Medicare requires providers to spend 20 minutes per month conducting treatment management—which must include live patient interaction—to bill for these services. Yet remote monitoring often largely involves patients collecting and care teams analyzing data outside of live interactions.

CMS should simplify billing requirements that create administrative burden without adding value and ensure requirements align with clinical need. As a first step, CMS should enable billing codes for fewer than 16 days of monitoring and 20 minutes of live patient interaction per month.

For additional details, see BPC's blog, From Promise to Best Practice: The Realities of Remote Patient Monitoring.

Technology Vendors, Data Providers, and Networks

1. Ecosystem

TD-1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

CMS should modernize its reimbursement policies to stimulate developer interest in building digital health products. As discussed in sections PR-1 and PA-5, Medicare billing requirements sometimes add unnecessary administrative burden, and reimbursement may not adequately account for the significant investments needed to establish a high-quality remote monitoring program. Developers consider past failures—such as FDA-approved products that went out of business due to limited uptake—when assessing market viability. Limited uptake due to reimbursement challenges may limit developer interest in building additional innovative solutions. CMS should eliminate geographic adjustments that reduce payments to providers serving rural and lower-cost areas, ensure reimbursement levels are adequate overall, and simplify billing requirements that create administrative burden without adding value.

CMS should also take steps to establish clear, predictable reimbursement pathways for high-value digital health products. Many of these products fail to fit into outdated benefit categories, creating uncertainty about whether and how CMS will reimburse them. Developers are hesitant to invest time and resources into innovation without clear expectations from CMS on coverage pathways. To address these issues, CMS could consider establishing a digital health "sandbox" program. Such a program could allow for early, non-binding engagement between CMS and developers, providing clarity on coverage pathways and real-world evidence expectations. Building on CMS's experience with sandbox models for data access and interoperability, this approach could offer a structured but flexible framework to accelerate access to safe and effective digital tools for Medicare beneficiaries.

2. Technical Standards and Certification

TD-6. What unique interoperability functions does TEFCA perform?

TEFCA creates a nationwide network that enables real-time clinical data exchange across previously siloed health information networks. Through a single common framework, TEFCA standardizes how participants verify identity, track data provenance, and exchange patient information, while supporting both query-based retrieval and push-based sharing of data. This allows for secure and scalable interoperability between health systems, payers, public health agencies, and other sectors.

For AI-enabled tools, TEFCA could enable access to high-quality, standardized clinical data from across care settings and geographies. However, access to this data is currently limited. Developers generally need to partner with TEFCA participants, such as providers or health systems, and can only use data for certain purposes, typically treatment, payment, and health care operations. Broader use for activities like AI model development are not generally permitted under current rules without data de-identification or patient authorization; future updates to the framework could explore opportunities to safely expand permissible uses.

3. Compliance

TD-19. Regarding price transparency implementation:
a. What are current shortcomings in content, format, delivery, and timeliness?
b. Which workflows would benefit most from functional price transparency?
c. What improvements would be most valuable for patients, providers, or payers, including CMS?
d. What would further motivate solution development?

Telehealth services, especially the practice of "incident to" billing—which allows nonphysician providers to bill under a supervising physician's national provider identification number—would benefit from greater price transparency and reporting. Before COVID-19, "incident to" billing enabled nonphysician providers—such as nurse practitioners or physician assistants—to bill under a supervising physician's higher rate, provided the physician was physically present in the same office. Since COVID-19, CMS has allowed for "virtual supervision," in which the physician is available online as needed. MedPAC has raised concerns about "incident to" billing—for telehealth and more broadly—noting that it makes it harder to monitor the quality of services.

To improve price transparency, CMS should phase out "incident to" billing for providers who can bill Medicare directly. Although BPC recommends eliminating "incident to" billing, a good first step would be for CMS to identify and monitor this type of billing.

CMS should also create a modifier or provider category to identify and monitor fully virtual providers in Medicare. Data on the prevalence and quality of fully virtual telehealth providers is limited because Medicare claims do not include information on a provider's employer or corporate affiliation. Notably, MedPAC estimates that 20% of behavioral health providers operated exclusively online in 2022. If feasible, CMS should also evaluate what portion of telehealth providers are fully virtual companies, versus individual practitioners who have shifted their practices to remote.

For additional details, see BPC's blog, Positioning Telehealth Policy to Ensure High-Quality, Cost-Effective Care.

Value-Based Care Organizations

1. Digital Health Adoption

VB-1. What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

Digital health tools, including AI-powered care navigation platforms, patient engagement apps, and remote monitoring technologies could help providers meet the goals of value-based care. However, accountable care organizations (ACOs) may hesitate to adopt these tools due to high upfront costs, challenges integrating new tools with existing electronic health record systems, limited reimbursement opportunities under traditional fee-for-service models, and uncertainty about the return on investment. ACOs serving low-income or rural populations may also face concerns about patient access to broadband services and digital devices, which can limit the effectiveness and reach of digital health management tools.

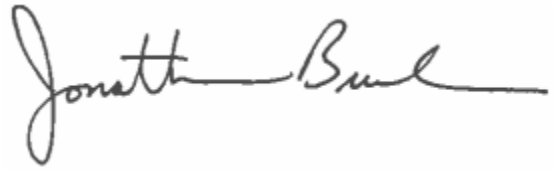
2. Technical Standards

VB-11. What specific interoperability challenges have you encountered in implementing value-based care programs?

In interviews with clinician stakeholders and policy experts, BPC heard that a major challenge to implementing and achieving success within value-based payment models is the persistent lack of EHR interoperability, such that EHR systems often cannot share data easily or seamlessly. In real terms, this hampers both care coordination across providers and access to the real-time feedback necessary to improve patients' outcomes and better control costs within alternative payment models. This persistent lack of EHR interoperability is compounded by misaligned quality measurement reporting requirements across payment models and payers, both of which further fragment data collection and increase clinician burden.

BPC's forthcoming report, Strengthening Primary Care: Medicare Physician Payment and Related Reforms, will further examine these challenges and offer federal policy solutions.

Sincerely,



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